Psychotherapy Research

ISSN: 1050-3307 (Print) 1468-4381 (Online) Journal homepage: https://www.tandfonline.com/loi/tpsr20

Early psychotherapy process and cluster B and C personality pathology: similarities and differences in interactions with symptomatic and interpersonal distress

Dr. Gregory G. Kolden , Marjorie H. Klein , Timothy J. Strauman , Sarah Chisholm-Stockard , Erin Heerey , Kristin L. Schneider & Tracey L. Smith

To cite this article: Dr. Gregory G. Kolden , Marjorie H. Klein , Timothy J. Strauman , Sarah Chisholm-Stockard , Erin Heerey , Kristin L. Schneider & Tracey L. Smith (2005) Early psychotherapy process and cluster B and C personality pathology: similarities and differences in interactions with symptomatic and interpersonal distress, Psychotherapy Research, 15:3, 165-177, DOI: 10.1080/10503300512331387825

To link to this article: https://doi.org/10.1080/10503300512331387825

Published online: 22 Feb 2007.

Submit your article to this journal

Article views: 157

View related articles

Citing articles: 1 View citing articles

Full Terms & Conditions of access and use can be found at
https://www.tandfonline.com/action/journalInformation?journalCode=tpsr20

Psychotherapy Research, July 2005; 15(3): 165–177

Routledge
Taylor & Francis Group

ORIGINAL ARTICLES

Early psychotherapy process and cluster B and C personality pathology: similarities and differences in interactions with symptomatic and interpersonal distress

GREGORY G. KOLDEN, MARJORIE H. KLEIN, TIMOTHY J. STRAUMAN, SARAH CHISHOLM-STOCKARD, ERIN HEEREY, KRISTIN L. SCHNEIDER, & TRACEY L. SMITH

University of Wisconsin—Madison Medical School

Abstract

In a prior study (Kolden & Klein, 1996), the authors found that the relationships between global personality pathology and early psychotherapy change processes (as defined by the Generic Model of Psychotherapy) were moderated by the extent of the patient's acute symptomatic and interpersonal distress. In the current study, the authors reanalyzed the same data to examine similarities and differences between personality disorder Clusters B (dramatic, emotional, or erratic) and C (anxious or fearful) in therapy process. In general, we found that more distressed patients reported greater defensiveness. There were no significant interactions between symptomatic distress and personality pathology in the prediction of any of the process variables. However, interpersonal distress moderated relationships between Clusters B and C and some therapy processes. Patients high in Cluster B felt more open and involved in the session when they were less distressed by their interpersonal problems at the start of therapy. In contrast, openness and insight were impeded among patients high in Cluster C when they were less distressed interpersonally. Therapists generally used more direct interventions and exploration of past experiences when working with patients higher in Cluster C pathology. However, therapists used direct interventions more specifically when patients with more severe Cluster B pathology were also higher in interpersonal distress. The discussion considers implications for the facilitation of productive early therapy process in patients suffering from Cluster B or C personality pathology.

This report is a reanalysis of data from a previous study (Kolden & Klein, 1996) that addressed the general question of whether Axis I–Axis II comorbidity impaired the process of psychotherapy in early sessions. In that study, we used a global, dimensional measure of personality pathology (i.e., Personality Assessment Form [PAF] ratings summarized across the 11 personality disorder dimensions contained in *Diagnostic and Statistical Manual of Mental Disorders*, third edition, revised; *DSM-III-R;* American Psychiatric Association [APA], 1987; Shea, Glass, Pilkonis, Watkins, & Docherty, 1987) and measures of symptomatic (Mental Health Index [MHI]; Howard, Lueger, Maling, & Martinovich, 1993) and interpersonal (Inventory of Interpersonal Problems [IIP]; Gurtman, 1996; Horowitz, Rosenberg, Baer, Ureño, & Villaseñor, 1988) distress at the start of therapy to predict patient and therapist processes early in therapy, as defined by measures derived from the Generic Model of Psychotherapy (Kolden, 1991, 1996; Kolden & Howard, 1992; Orlinsky, Grawe, & Parks, 1994; Orlinsky & Howard, 1986a, 1987; Orlinsky, Ronnestad, & Willutzki, 2004; Saunders, Howard, & Orlinsky, 1989). This investigation was based on the premise that the reports of poorer therapy outcomes for patients with comorbid Axis I and Axis II disorders (see reviews by Mulder, 2002; Shea, Widiger, & Klein, 1992; Tyrer, Gunderson, Lyons, & Tohen, 1997) might be explained in part by the challenges that personality pathology

The authors are grateful to Ken Howard for providing access to data from the Northwestern Research Project on Long-Term Psychotherapy and for clarifying feedback. This work was partially supported by National Institute of Mental Health Research Grant R01 MH42901 (1988–1993) and by awards from the National Alliance for Research on Schizophrenia and Depression and the University of Wisconsin—Madison Graduate School and Department of Psychiatry Research and Development Fund.
Correspondence: Dr. Kolden, Department of Psychiatry, 6001 Research Park Blvd., Madison, WI 53719. E-mail: ggkolden@wisc.edu

ISSN 1050-3307 print/ISSN 1468-4381 online © 2005 Society for Psychotherapy Research
DOI: 10.1080/10503300512331387825

presents to the therapeutic process (e.g., Andreoli, Gressot, Aapro, Tricot, & Gagnolons, 1989; Beck, Freeman, & associates, 1990; Wagner, Riley, Schmidt, McCormick, & Butler, 1999). It is also possible that features of some personality disorders, such as dependence, might actually promote higher levels of treatment compliance (Ashton, Rawlins, & Tyrer, 1990). In either case, positive therapy outcomes may be more likely to occur when treatment is appropriately modified to address personality pathology (Barber & Muenz, 1996; Hardy et al., 1995; Hoffart & Martinsen, 1993; Kraus & Reynolds, 2001; Monson, Odland, Faugli, Daae, & Eilertsen, 1995; Tyrer et al., 1997; Winston, Winston, Samstag, & Muran, 1994).

Our prior results highlighted the importance of considering both personality pathology and acute distress in the prediction of early therapy processes. When working with patients with more severe overall personality pathology, therapists reported that they provided more explicit direction and structure when these patients were also more distressed by their interpersonal problems. From the patient's perspective, personality pathology combined with interpersonal distress was associated with a greater sense of insight and learning (i.e., Realizations).[1] However, interpersonal distress interfered with Session 3 patient reports of Openness. We concluded that personality pathology must be considered together with interpersonal distress when selecting effective treatment strategies for personality-disordered patients.

Clinical experience and theory suggest that different issues and challenges are presented by patients with different kinds of personality pathology. Thus, in the current study, we have elaborated on our earlier findings by differentiating the effects of the two personality disorder clusters, Cluster B (dramatic, emotional, or erratic) and Cluster C (anxious or fearful), that were most prevalent in the sample. Although both clusters share features that may complicate therapy, we also hypothesize that each presents somewhat different challenges to the establishment of the therapeutic alliance and progress in early sessions. Personality disorders have traditionally been grouped into clusters because of similarities in descriptions and diagnostic criteria (e.g., APA, 1987). Although the cluster approach has been subject to some debate (Blashfield & McElroy, 1995), it has been confirmed by several factor analytic studies (Hyler & Lyons, 1988; Hyler et al., 1990; Kass, Skodol, Charles, Spitzer, & Williams, 1985), and studies using the cluster-level classification continue to appear in the literature (e.g., Gude, Monsen, & Hoffart, 2001; Kraus & Reynolds, 2001). We have chosen to use the cluster organizational scheme in the current analyses because some of the individual personality disorders were infrequently present in our sample.

Research as well as clinical experience demonstrates that Cluster B personality pathology, which encompasses narcissistic, histrionic, borderline, and antisocial disorders, is especially difficult to treat (e.g., Kraus & Reynolds, 2001). As a group, Cluster B patients tend to be emotionally volatile and often express intense negative affect in sessions. They are likely to externalize the source of their problems and find it difficult to consider their personal contribution to and responsibility for managing them. Moreover, they tend to demand attention from the therapist. In contrast, patients with the disorders encompassed by Cluster C (i.e., avoidant, dependent, obsessive–compulsive, and passive–aggressive) share a reticence in relationships and often find it difficult to openly express their feelings and needs in therapy sessions (Gude et al., 2001). Although they may desire close relationships, they are inhibited by fearfulness, low self-esteem, and concerns about performance.

As in our earlier work, we used the Generic Model of Psychotherapy to define the psychotherapy change processes (see later and Orlinsky et al., 2004). Considering the differences between Cluster B and Cluster C pathology, and differentiating the role of symptomatic and interpersonal distress in the therapy sessions, we present four hypotheses.

Hypothesis 1: Cluster B personality disorders are characterized by the propensity to express distress (resulting from vulnerability to emotional dysregulation) and to act on feelings of entitlement in relationships. Therefore, when patients with higher levels of Cluster B pathology are less symptomatically distressed, they will report better process as measured by the Bond, Openness, and Realization Scales. In contrast, these patients will also report better process when they are more distressed by interpersonal problems.

Hypothesis 2: Cluster C personality disorders are characterized by reticence in relationships and by constriction in the experience and open expression of feelings. Therefore, patients with higher levels of Cluster C pathology will have better therapy process when they are less distressed by interpersonal problems. However, the process for these patients will be enhanced when symptomatic distress is higher. In other words, the patterns shown by high Cluster C patients will be opposite from those shown by high Cluster B patients.

Hypothesis 3: To foster regulation of affect in patients high in Cluster B, therapists will use direct interventions (as measured by the Prescriptive Operations Scale) more frequently than for other

patients. This strategy will be used most frequently when patients high in Cluster B are also more symptomatically distressed.

Hypothesis 4: To overcome the emotional constriction that is characteristic of Cluster C, therapists will focus more frequently on patient experiencing, as measured by the Experiential Interventions Scale, for these patients than for other patients. This strategy will be used most frequently when patients high in Cluster C are also more distressed by their symptoms or interpersonal problems.

Method

Participants

Participants were outpatients who underwent individual psychotherapy at the Northwestern University Institute of Psychiatry as part of the Long-Term Therapy Research Project (LTTRP; Howard, Orlinsky, & Lueger, 1995). The target sample for the current analyses was the sample of LTTRP participants (a) who had completed at least three therapy sessions (when process ratings were made), (b) whose therapist had completed the PAF describing their personality disorders dimensionally, (c) who had completed either the MHI or the IIP at the beginning of therapy, and (d) whose process measures had been completed, by either the patient or the therapist, at the end of the third session. These criteria yielded four sets of cases for analyses with process ratings as dependent variables: (a) 127 cases for analyses of patient process ratings with MHI and PAF as predictors; (b) 148 cases for therapist ratings of their interventions with the MHI and PAF as predictors; (c) 140 cases for analyses of patient process ratings with IIP and PAF as predictors; and (d) 152 cases for therapist ratings of their interventions with the IIP and PAF as predictors. Table I summarizes the demographic characteristics of these four groups and indicates the extent of overlap between the groups.

Axis I diagnoses were made by intake clinicians after a semistructured admission interview. Diagnoses for Axis II were derived from the PAF completed by the intake clinicians. Extensive Axis I–Axis II comorbidity was exhibited in the four groups; 66–71% received both an Axis I and an Axis II diagnosis. Although a dimensional approach to assessing Cluster B and Cluster C pathology was used in the current study, PAF ratings of 4 or greater on a 6-point scale were also used to approximate "categorical" diagnoses (see later for a description of the PAF). As Table I indicates, within the four groups used for the process analyses, 21% to 30% of the patients received one or more Cluster B diagnoses and 44% to 46% received one or more Cluster C diagnoses. Diagnoses in both clusters were present for 9% to 14%. Cluster A diagnoses were present for 9% to 13% of the patients. This sample appears to be fairly typical of the psychotherapy outpatient population in terms of demographics, comorbidity, and the prevalence of personality pathology (Pfohl, Black, Noyes, Coryell, & Barrash, 1990; Taube, Kessler, & Feuerberg, 1984; Vessey & Howard, 1993).

Therapists

Seventy-nine therapists participated in this study; 63 (80%) were psychologists, ten (13%) were psychiatrists, and six (8%) were social workers. Seventy-two (92%) were between 22 and 39 years of age, 51 (65%) were female, and 26 (38%) were married. Sixty-nine (87%) of the therapists had undergone personal therapy, and 38 (48%) had seen more than ten patients in individual psychotherapy under supervision. Thus, most were in an advanced stage of training. Their dominant theoretical orientation was psychodynamic; supervisors typically espoused this approach and case conceptualizations and presentations followed this model, although no treatment manuals were explicitly used. As such, this study represents a naturalistic examination of Cluster B and Cluster C personality pathology and change processes in dynamic therapy as delivered in a clinical service and training setting.

Measures

Except for the PAF, instruments were scored such that higher scores indicate less pathology or distress, more optimal patient process, or greater frequency of therapist operations. Raw scores were transformed into *T* scores with means of 50 and standard deviations of ten, using the full LTTRP sample norms.

PAF (Shea et al., 1987). The PAF is a clinician-rated measure of personality pathology in which prototypic descriptions are rated on 6-point dimensional scales for each of the 11 personality disorder categories contained in the *DSM-III-R* (APA, 1987). When used categorically, a personality disorder is considered present if a score of four (to a considerable extent) or more is assigned. The overall reliability of this categorical cutoff score has yielded a kappa of .48. Pilkonis, Heape, Ruddy, and Serrao (1991) reported an overall interrater reliability kappa of .36 using intake data, .44 using follow-up data, and .49 when knowledgeable informants were interviewed. In this study, Cluster A, B, and C (PAF-A,

Table I. Characteristics of patients in the four analysis samples.

Variable	MHI analyses		IIP analyses	
	Patient ratings	Therapist ratings	Patient ratings	Therapist ratings
N	127	148	140	152
Age (years [*M, SD*])	32 (8.1)	31 (8.3)	32 (8.0)	32 (8.6)
Gender (*n*, %)				
Female	90 (71)	105 (71)	96 (69)	108 (71)
Male	37 (29)	43 (29)	44 (31)	44 (29)
	MHI/PD analyses		IIP/PD analyses	
Marital status (*n*, %)				
Single/never married	79 (62)	94 (64)	79 (63)	89 (65)
Married/remarried	26 (21)	30 (20)	27 (21)	28 (20)
S/D/W	22 (17)	24 (16)	20 (16)	21 (15)
Education (*n*, %)				
≤High school or less	12 (10)	18 (12)	13 (10)	18 (13)
At least some college	33 (26)	35 (24)	32 (25)	31 (23)
Graduate/professional	82 (65)	95 (64)	81 (64)	89 (64)
Employment status (*n*, %)				
Not employed	16 (13)	23 (16)	13 (10)	18 (13)
Part or full time	111 (87)	124 (84)	113 (90)	119 (87)
Diagnostic status (*n*, %)				
Any Axis I	122 (96)	141 (96)	134 (96)	145 (96)
Any Axis II	88 (69)	104 (71)	96 (69)	111 (73)
Axis I & II	86 (68)	100 (68)	93 (66)	107 (71)
Any Cluster A	14 (11)	19 (13)	12 (9)	16 (11)
Any Cluster B	27 (21)	44 (30)	27 (19)	42 (28)
Any Cluster C	58 (46)	67 (45)	61 (44)	67 (44)
Both B and C	12 (9)	20 (14)	12 (9)	19 (13)

Note. There are 66 cases in the Mental Health Index (MHI) analyses with both patient and therapist ratings; 71 cases in the Inventory of Interpersonal Problems (IIP) analyses had both patient and therapist ratings; 66 cases of therapist ratings were in both MHI and IIP analyses; 120 cases of patient ratings were in both MHI and IIP analyses.

PAF-B, PAF-C, respectively) dimensional scores were derived by summing ratings over the appropriate categories. Cluster B scores were composed of ratings for antisocial, borderline, histrionic, and narcissistic; Cluster C scores summarized ratings of avoidant, dependent, obsessive–compulsive, and passive–aggressive disorders. Cluster A scores were composed of ratings for paranoid, schizoid, and schizotypal disorders. Alphas for the Cluster scores in the current study were .67 for Cluster A, .73 for Cluster B, and .57 for Cluster C.

A factor analysis of the PAF ratings in this sample yielded three factors that accounted for 55% of the variance. Except for the passive–aggressive dimension, all other dimensions had loadings of .40 or greater on only one factor. The first factor was composed of members of Cluster A (i.e., paranoid, schizoid, schizotypal, as well as antisocial; eigenvalue = 3.22, 29% of variance). The second factor was composed of three members of Cluster B (i.e., histrionic, narcissistic, and borderline; eigenvalue = 1.65, 15% of variance). The third factor was composed of three members of Cluster C (i.e., dependent, avoidant, and obsessive–compulsive; eigenvalue = 1.18, 11% of variance). When the Cluster A disorders were omitted from the factor analysis, two factors remained, accounting for 50% of the variance. Antisocial loaded on the factor with the other Cluster B dimensions (eigenvalue = 2.35, 29% of the variance) and passive–aggressive loaded on the second factor with other Cluster C disorders (eigenvalue = 1.62, 20% of variance) consistent with the conception proposed in *DSM-III-R* (APA, 1987).

MHI (Howard et al., 1993). The MHI is a multidimensional, patient-rated assessment of mental health status made up of three components: Well-Being, Current Symptoms, and Current Life Functioning. An internal consistency coefficient of .81 has been demonstrated for the MHI. Patients completed the ratings before the first therapy session.

IIP. The IIP is a 26-item measure of current distress arising from interpersonal sources (adapted

from Horowitz et al., 1988). The shortened version of the IIP used in this study demonstrated an internal consistency of .88. As noted in Gurtman (1996), this abbreviated version of the IIP consisted of 26 items chosen on the basis of a factor analysis of the IIP and included the three to five highest-loading items from the six problem categories of the IIP originally described by Horowitz et al. (1988). The general factor score from the 26-item IIP correlated .95 with the mean of the 127 items from the original IIP (Gurtman, 1996). Patients completed the IIP before the first therapy session.

Therapy Session Report (TSR). The patient version of the TSR (see Orlinsky & Howard, 1986b, for a review of the development and application of this instrument) provided the measures for the three patient-reported process variables used in this study: Therapeutic Bond (e.g., patient's rating of empathy, attentiveness, understanding, collaboration, etc. in the patient–therapist relationship), Therapeutic Openness (e.g., patient's rating of satisfaction with his or her behavior in the session, feeling free and spontaneous in self-expression), and Therapeutic Realizations (e.g., patient's rating of insight, unburdening, and feelings of encouragement). TSRs were obtained from patients after the third session of psychotherapy. Internal consistency for these scales has been established: .62 for Bond (Saunders et al., 1989), .69 for Openness, and .86 for Realizations (Kolden, 1991). These scales have been shown to have predictive validity for early change in mental health status (Kolden, 1996; Kolden, Strauman, Halverson, & Heerey, 1997) and for termination outcome (Kolden, 1991; Kolden & Howard, 1992).

Therapeutic Procedures Inventory–Revised (TPI-R; Orlinsky, Lundy, Howard, Davidson, & O'Mahoney, 1987). The TPI-R is a therapist-rated assessment of the frequency of interventions used in therapy sessions. Three scales, Prescriptive Operations (e.g., explicit guidance, behavioral interventions), Exploratory: Past-focused Operations (e.g., exploration of childhood experiences, discussion of patterns or themes in patient's experience), and Exploratory: Experiential Operations (e.g., clarifying patient's feelings, helping patient express feelings) were derived from factor analysis with internal consistencies of .82, .74, and .63, respectively (McNeilly & Howard, 1991). Therapists were asked to rate their use of these interventions after the third therapy session.

Data analysis

Multiple regression with listwise missing-variable treatment was used to examine the associations of Cluster B or C personality disturbance, symptomatic (MHI) or interpersonal distress (IIP), and their interactions, with Session 3 patient and therapist process reports. To focus the analyses specifically on the impact of Cluster B or C pathology, PAF-A scores were entered as controls in each analysis because of this cluster's moderate correlations with Clusters B and C (Table II). Interaction terms for Cluster B or C personality pathology by symptomatic or interpersonal distress were entered in four separate analyses (i.e., one each for PAF-B × MHI, PAF-B × IIP, PAF-C × MHI, and PAF-C × IIP). As recommended by Aiken and West (1991), all data were centered before interaction terms were computed. We chose not to enter both interaction terms in a single model on the grounds that this

Table II. Correlates of session 3 psychotherapy process.

Variable	1	2	3	4	5	6	7	8	9	10	11
Personality pathology and symptomatic and interpersonal distress											
1. PAF-A	—	.52***	.30***	−.15*	−.09	−.07	−.05	−.13	.01	.08	.07
2. PAF-B		—	.16***	−.12*	−.11*	.09	.09	.04	.08	.04	.09
3. PAF-C			—	−.12*	−.14*	−.12	−.11	−.10	.16*	.23***	.07
4. MHI				—	.55***	−.07	.20*	.01	−.01	−.08	−.02
5. IIP					—	.07	.32***	−.04	−.02	.04	.07
Patient therapy session report											
6. Bond						—	.49***	.73***	−.03	−.02	.25*
7. Openness							—	.30***	−.01	.10	.10
8. Realizations								—	−.02	−.01	.10
Therapist-rated therapeutic procedures											
9. Prescriptive									—	.23***	.27***
10. Past-Focused										—	.34***
11. Experiential											—

Note. Personality Assessment Form = PAF; Mental Health Index = MHI; Inventory of Interpersonal Problems = IIP.
*$p < .05$. **$p < .01$. ***$p < .001$.

approach would only reveal differential Cluster B and C effects and not identify instances in which the interaction patterns were similar for these two aspects of personality disorder.

The interpretation of interaction effects in multiple regression requires post hoc statistical procedures akin to post hoc comparisons of mean differences in analysis of variance (Cohen & Cohen, 1983). Briefly, graphs are generated with levels of the dependent/criterion variables on the *y*-axis (in this case, therapy process variables) as predicted by low (1 *SD* below the mean) and high (1 *SD* above the mean) scores for each independent variable. The significance of the simple slopes of the associations between process and personality pathology for each graph was determined according to regression procedures provided by Aiken and West (1991).

Results

Table II summarizes the zero-order correlations among Session 3 process variables, personality disorder ratings (PAF-B, PAF-C, and PAF-A), the MHI, and the IIP. Cluster B personality pathology was not directly associated with any of the patient- or therapist-reported process variables. Cluster C pathology was positively correlated with therapist-reported use of Prescriptive and Exploratory Past-Focused interventions. There was a positive association between the patient's ratings of their Openness with both measures of distress (i.e., MHI and IIP), consistent with our previous study (Kolden & Klein, 1996). Patients with less distress at the beginning of therapy rated themselves as more open and involved in the therapeutic enterprise by the third session than those with greater distress. The three patient-rated process variables were significantly intercorrelated, as were the three therapist-rated intervention variables.[2]

Cluster B and C personality pathology, global distress, and therapy process

We first conducted regression analyses focused on personality clusters, global distress, and personality–distress interactions, separately for Clusters B and C. The only interaction that reached significance was the interaction between initial symptomatic distress (MHI) and Cluster B for the therapist's reported use of Prescriptive Operations in the third session (not included in table). Neither of the simple slopes for this interaction reached significance, although there was a trend ($p = .06$), indicating that the therapist's use of Prescriptive Operations increased in relation to the amount of patient Cluster B pathology when these patients were more distressed by symptoms at the beginning of therapy.

Cluster B and C personality pathology, interpersonal distress, and therapy process

Table III summarizes the results of the four regression analyses examining the relationships between Cluster B and C personality disturbance and distress resulting from interpersonal problems that yielded significant personality by IIP interaction effects. Significant interactions effects are illustrated and discussed next.

Cluster B personality pathology and therapy process. Interaction effects for Cluster B with the IIP were apparent for one patient-rated process variable—Openness—and one therapist-rated intervention—Prescriptive Operations. When distress from interpersonal problems was lower at the beginning of therapy, more severe Cluster B personality pathology was associated with greater patient Openness in the third session (the simple slope for patients with low distress was significant; see Table III). As illustrated

Table III. Results of multiple regression with cluster B and C personality pathology by initial IIP status on session 3 patient and therapist process ratings.

							t Simple slope	
Process variable	R^2	β_{IIP}	β C1 A	β C1 B	β C1 C	β IIP × PD	High distress	Low distress
Cluster B × IIP interactions								
Openness	.17***	−0.66	−0.12	−0.68	−0.05	1.27*	0.46	3.26***
Prescriptive	.09*	1.38**	−0.13	1.49**	0.21*	−1.84**	3.05**	−1.32
Cluster C × IIP interactions								
Realizations	.07+	0.75	−0.15	0.12	0.70	−1.02*	0.43	−2.10**
Openness	.18***	1.38***	−0.07	0.23*	1.01**	−1.32**	1.45	−2.22*

Note. Inventory of Interpersonal Problems = IIP.
+$p < .10$. *$p < .05$. **$p < .01$. ***$p < .001$.

in Figure 1, the significant slope for patients with less interpersonal distress indicates that patients high in Cluster B pathology felt more open in the therapy session when they were less distressed by their interpersonal problems.

When distress from interpersonal problems was higher initially, greater Cluster B personality pathology was associated with more frequent therapist use of Prescriptive Operations in the third session (the simple slope for patients with high interpersonal distress was significant; see Table III). As Figure 2 illustrates, among patients with higher interpersonal distress, the greater the Cluster B personality pathology, the more frequently the therapists were engaged in Prescriptive Operations in the third session.

Cluster C personality pathology and therapy process. A distinctly different pattern was found for the interaction of Cluster C personality pathology and interpersonal distress. For Openness, the direction of the Cluster C relationship was the reverse of that seen for Cluster B. When interpersonal distress was lower at the start of therapy, more severe cluster C pathology was associated with less patient Openness in the third session (the simple slope for patients with low distress was significant; see Table III and Figure 3). The pattern described previously was similar for Therapeutic Realizations. When distress over interpersonal problems was low, greater Cluster C pathology was associated with fewer Realizations (the simple slope for patients with low distress was significant; see Table III and Figure 4).

Discussion

Given the dimensional method of personality assessment used in this study, it is important from the outset to recognize that this work only involves an examination of the general roles of Cluster B or C in psychotherapy. It is not a study of Cluster B or C personality-disordered patients per se; because the clusters often overlap, the results should be viewed as an examination of the effects that the different concerns that characterize the two clusters have during early sessions of therapy.

Table IV provides a summary of the results of the current study. Although the main effect findings lend themselves to a fairly straightforward interpretation, the results of interaction effects are always a challenge to convey. To ease this burden for our readers, we have adopted the convention of consistently

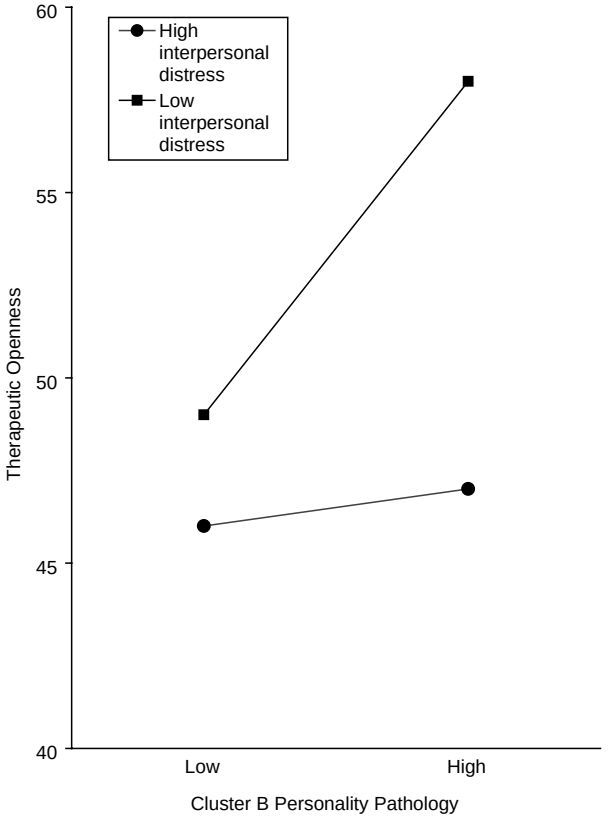

Figure 1. Interaction between Cluster B personality pathology and interpersonal distress (IIP) for Therapeutic Openness; simple slope significant for low distress.

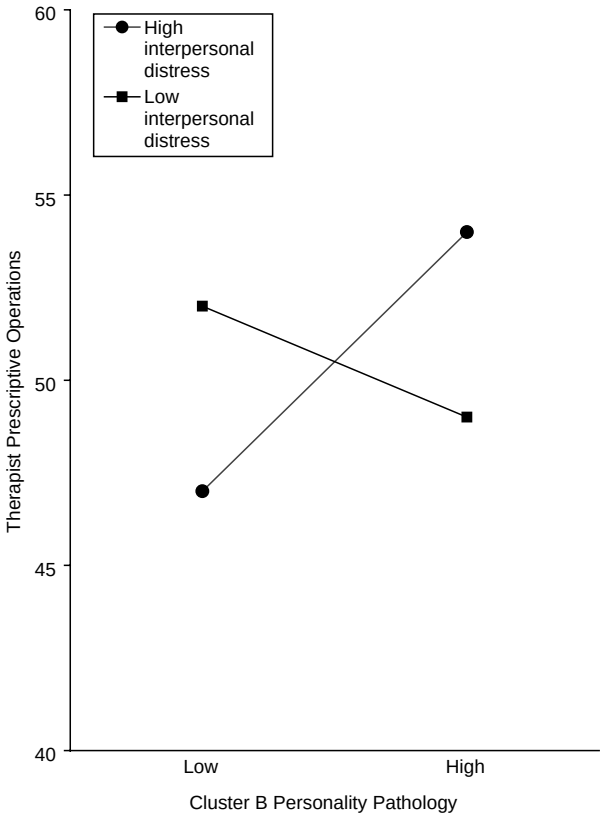

Figure 2. Interaction between Cluster B personality pathology and interpersonal distress (IIP) for Therapist Prescriptive Operations; simple slope significant for high distress.

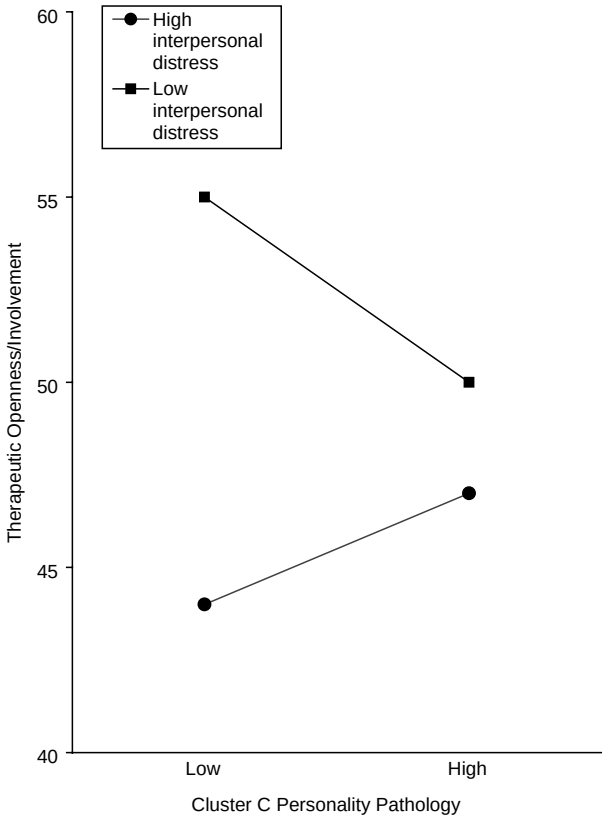

Figure 3. Interaction between Cluster C personality pathology and interpersonal distress (IIP) for Therapeutic Openness; simple slope significant for low distress.

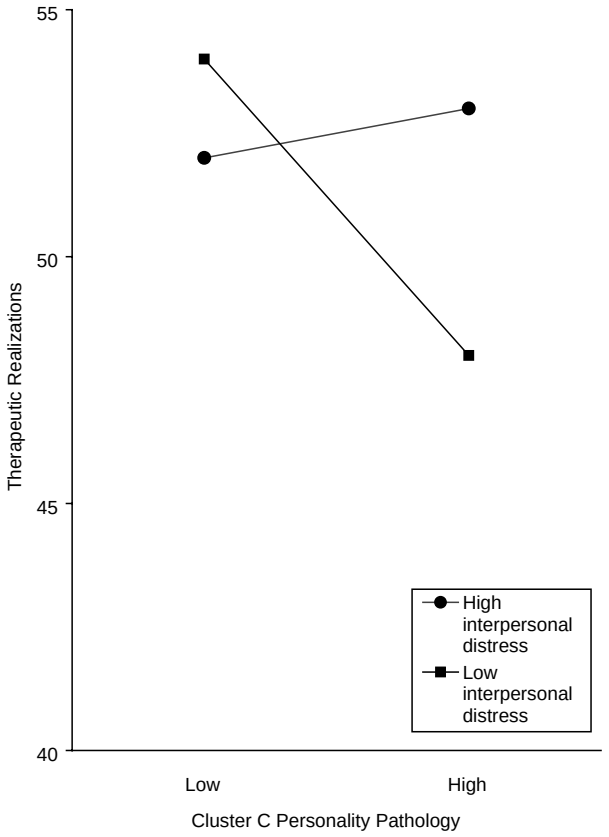

Figure 4. Interaction between Cluster C personality pathology and interpersonal distress (IIP) for Therapeutic Realizations; simple slope significant for low distress.

presenting the interaction effects in terms of the relationship between personality pathology and therapy process for high and low levels of symptomatic or interpersonal distress.

Greater defensiveness (i.e., less openness) by the third therapy session was associated with both symptomatic and interpersonal distress at the beginning of therapy, independent of the extent of personality pathology. Essentially, severe initial distress seems to interfere with patient engagement and involvement in the early stages of the therapeutic enterprise. Although symptomatic distress was associated with patient defensiveness, there were no significant relations between personality pathology

Table IV. Summary of significant correlations and interactions.

Independent Variable	Dependent Variable	
Cluster C high	Higher therapist Prescriptive Operations Higher therapist Exploratory: Past-Focused Operations	
Openness high	Lower symptomatic distress (MHI) Lower interpersonal distress (IIP)	
Level of patient distress	Patient personality pathology	Therapy process
Interaction effects for Cluster B		
Symptomatic distress high	Cluster B high	Higher therapist Prescriptive Operations (trend)
Interpersonal distress high	Cluster B high	Higher therapist Prescriptive Operations
Interpersonal distress low	Cluster B high	Higher patient Openness
Interaction Effects for Cluster C		
Interpersonal distress low	Cluster C high	Lower patient Openness
Interpersonal distress low	Cluster C high	Lower patient Realizations

Note. Mental Health Index = MHI; Inventory of Interpersonal Problems = IIP.

and the other therapy process ratings when considering this type of distress. However, when the distress was interpersonal, the relations between Openness and personality pathology differed according to the type of personality pathology. Specifically, when interpersonal distress was lower at the start of therapy, higher Cluster B personality pathology was associated with greater patient-rated Openness in the third session. In contrast, when interpersonal distress was lower, more severe Cluster C pathology was associated with the patients rating themselves as less open and having fewer insights or sense of unburdening in the session (i.e., Realizations). Lower interpersonal distress seemed to facilitate the patient's sense of Openness early in therapy among patients high in Cluster B, whereas Openness was impeded when patients higher in Cluster C had fewer interpersonal problems at the start of therapy. These findings do not support Hypothesis 1 for Cluster B patients but do support Hypothesis 2 for Cluster C patients.

Patients with Cluster B personality disorders are vulnerable to emotional dysregulation, are generally not reticent about expressing or acting out their feelings in relationships, and may often demand attention from therapists. Cluster B personality disturbance alone, however, did not appear to interfere with the patient's perceptions of the therapy processes early in dynamic treatment. On the other hand, those patients who felt the most open early in therapy were higher in Cluster B pathology and lower in initial interpersonal distress. It appears that the issues brought to therapy by those with more severe Cluster B personality pathology may be discussed more openly and less defensively when interference from interpersonal distress is at a minimum. For example, when a patient with more severe borderline pathology is less overtly distressed by fundamental relationship issues, he or she may be more aware of or able to talk more openly in therapy about these issues.

In contrast, we found that patients with more severe Cluster C pathology felt less engaged and benefited less in Session 3 (i.e., less Openness and fewer Realizations) when interpersonal distress was lower at the outset of therapy. To the extent that Cluster C pathology is associated with reticence on the patient's part to engage in therapy, whether from fear of rejection or performance concerns, it appears that, absent some interpersonal distress, the presence of Cluster C pathology is also associated with less involvement and insight. We also found that Cluster C pathology in general was associated with the therapist's more frequent use of directive interventions and exploration of past experiences (i.e., Prescriptive and Past-Focused Operations). Given the interpersonal fearfulness associated with Cluster C, therapists may need to provide more explicit guidance for these patients regardless of the level of distress experienced. This kind of sensitivity to differential patient needs in the two clusters may also explain the finding that therapists used directive interventions consistently when Cluster C pathology was high but used them more selectively when faced with elevated Cluster B pathology. That is, therapists described themselves as being more active and directive when patients high in Cluster B pathology were also experiencing greater interpersonal distress. Considering this result together with the finding that patients higher in Cluster B pathology feel more open in therapy when their interpersonal distress is low, we might speculate that when patients high in Cluster B pathology are less distressed and more involved in the early sessions, therapist direct intervention is not required to contain their distress. In contrast, a more direct level of therapist intervention may be consistently needed for the more avoidant and inhibited Cluster C individuals.

Although patient reports of interpersonal distress moderated the relationships between personality pathology and therapy process, no such interactions were found when symptomatic distress was measured by the MHI. This suggests that symptomatic distress is not as specifically relevant to the early therapy process with personality-disordered patients as the interpersonal concerns assessed by the IIP. This underscores the prominence of the interpersonal context in the conceptualization and treatment of the personality disorders, as suggested, for example, by Benjamin (1996), Millon (1986), and others (see Millon & Klerman, 1986, or Livesley, 1995, for reviews).

Limitations

We wish to note the limitations of our findings. First, it is important to bear in mind that the process measures consisted of the patients' and therapists' reports of session events without independent confirmation. However, the process variables examined in this study, especially for the patient, are highly phenomenological in nature and may be best captured by self-report (Kolden, 1996). Second, this was a naturalistic study of psychodynamic therapy as delivered in a training clinic; the therapy was not manualized. Thus, the current study falls in a middle ground between a controlled clinical trial on one end of the spectrum and a study of outpatient psychotherapy/treatment as usual on the other and is thus generalizable to actual practice. Manuals were used as guides for training and supervision; however, formal fidelity–adherence ratings were not con-

ducted. Third, although these findings may be of limited utility for understanding processes in the later stages of therapy or longer term treatment outcome, they are important for understanding the processes involved in change early in therapy, the period in which the greatest rate of change occurs (Howard, Kopta, Krause, & Orlinsky, 1986; Lambert, Hansen, & Finch, 2001). Moreover, attention to therapy process during the early sessions of therapy is crucial because attrition is most likely to occur at this time, especially among patients suffering from personality pathology (e.g., Kraus & Reynolds, 2001; Merrill, Tolbert, & Wade, 2003; Persons, Burns, & Perloff, 1988). The use of the PAF for assessing personality pathology is another limitation. The PAF consists of analogue scales and yields dimensional scores that map onto the *DSM-III-R* personality disorder descriptions (Shea et al., 1987). Finally, there are limitations to the use of the clustering approach to the personality disorders, primarily because the structure of the clusters has not been consistently validated. Yet our factor analysis of the PAF provides some support for the cluster approach, and the substantive findings provide support for the usefulness of considering differences between clusters when engaging in treatment conceptualization and planning. We also note that our choice to group patients at the cluster level limits the generalizability of our findings to any specific personality disorder (i.e., they may not apply equally to the treatment of specific, single disorders within the clusters). Nonetheless, given the distinct findings obtained for Cluster B versus C pathology, respectively, we believe that our results illustrate a critical aspect of productively engaging patients with complex problems in psychotherapy.

Notes

[1] In our previous study (Kolden & Klein, 1996), in which we considered all personality disorders combined, we used a pairwise missing data treatment for correlational and hierarchical multiple regression analyses. For the current study, in which we examine Clusters B and C separately, we have elected to use a listwise missing-value treatment. To make direct comparisons between our prior findings and the current results, we have omitted mention of interaction findings from the earlier analyses that failed to reach significance when reanalyzed with a listwise approach. Specifically, in Kolden and Klein (1996), we found that overall personality disorder pathology in conjunction with greater symptomatic and interpersonal distress was associated with patient reports of better Bonds and that Realizations were also greater in patients in whom greater personality pathology was associated with greater symptomatic distress. These findings did not reach significance when reanalyzed using listwise deletion of missing data.

[2] It is typical for relationships among constructs from similar rating perspectives to be greater than those between different rating perspectives. For example, one would generally expect a greater association among patient-rated therapy processes than between patient- and therapist-rated processes. Regardless, the correlation of .73 between Bond and Realizations scales undoubtedly raises concerns about whether these two scales are assessing a similar underlying construct as opposed to unique therapy processes. We argue that Bond and Realizations represent distinct constructs that have been theoretically derived from different lines of research. Bond is akin to conceptions of the facilitative conditions (Rogers, 1957) or therapeutic alliance (Horvath & Bedi, 2002) and is centered in the patient–therapist relationship (Orlinsky & Howard, 1986a). Realization is akin to conceptions of the patient's sense of gain in the session through the achievement of insight, conflict resolution, or improved well-being. In the Generic Model (Orlinsky & Howard, 1987), Realizations can have the effect of strengthening the Bond, but Realizations depend on therapeutic interventions and the patient's Self-Relatedness. The item content of the two scales taps these distinctive conceptions of the therapy process. Bond items inquire about patients' sense of their relationship with the therapist in terms of friendliness and warmth, understanding, or mutual collaboration (e.g., "Was your therapist friendly and warm toward you?", "How well did your therapist seem to understand what you were feeling and thinking?", or "Were you able to focus on what was of real concern to you?"). Realizations items inquire about what the patients got out of the session for themselves, such as sense of unburdening, gaining insight, or feeling encouraged (e.g., "A chance to let go and get things off my chest" or "More understanding of the reasons behind my behavior and feelings"). Prior published studies (Kolden, 1991, 1996; Kolden & Howard, 1992) have empirically demonstrated that Bond and Realizations uniquely account for significant variance in Session Progress. For these reasons, we believe that the Bond and Realizations scales represent distinct constructs and have included them as dependent variables in separate analyses in this study

References

Aiken, L. S., & West, S. G. (1991). *Multiple regression: Testing and interpreting interactions*. Newbury Park, CA: Sage.

American Psychiatric Association. (1987). *Diagnostic and statistical manual of mendal disorders* (3rd ed., rev.). Washington, DC.

Andreoli, A., Gressot, G., Aapro, N., Tricot, L., & Gagnalons, M. Y. (1989). Personality disorders as a predictor of outcome. *Journal of Personality Disorders*, *3*, 307–320.

Ashton, C. H., Rawlins, M. D., & Tyrer, S. P. (1990). A double-blind placebo-controlled study of buspirone in diazepam withdrawal in chronic benzodiazepine users. *British Journal of Psychiatry*, *157*, 232–238.

Barber, J. P., & Muenz, L. R. (1996). The role of avoidance and obsessiveness in matching patients to cognitive and interpersonal psychotherapy: Empirical findings from the Treatment for Depression Collaborative Research Program. *Journal of Consulting and Clinical Psychology*, *64*, 951–958.

Beck, A. T., Freeman, A., & associates. (1990). *Cognitive therapy of personality disorders*. New York: Guilford Press.

Benjamin, L. S. (1996). *Interpersonal diagnosis and treatment of personality disorders* (2nd ed.). New York: Guilford Press.

Blashfield, R. K., & McElroy, R. A. (1995). Confusions in the terminology used for classificatory models. In W. J. Livesley (Ed.), *The DSM-IV personality disorders* (pp. 407–461). New York: Guilford Press.

Cohen, J., & Cohen, P. (1983). *Applied multiple regression/correlation analysis for the behavioral sciences* (2nd ed.). Hillsdale, NJ: Erlbaum.

Gude, T., Monson, J. T., & Hoffart, A. (2001). Schemas, affect consciousness, and cluster C personality pathology: A prospective one-year follow-up study of patients in a schema-focused

short-term treatment program. *Psychotherapy Research*, *11*, 85–98.

Gurtman, M. B. (1996). Interpersonal problems and the psychotherapy context: The construct validity of the Inventory of Interpersonal Problems. *Psychological Assessment*, *8*, 241–255.

Hardy, G. E., Barkham, M., Shapiro, D. A., Stiles, W. B., Rees, A., & Reynolds, S. (1995). Impact of cluster C personality disorders on outcomes of contrasting brief psychotherapies for depression. *Journal of Consulting and Clinical Psychology*, *63*, 997–1004.

Hoffart, A., & Martinsen, E. W. (1993). The effect of personality disorders and anxious-depressive comorbidity on outcome in patients with unipolar depression and with panic disorder and agoraphobia. *Journal of Personality Disorders*, *7*, 304–311.

Horowitz, L. M., Rosenberg, S. E., Baer, B. A., Ureño, G., & Villaseñor, V. S. (1988). Inventory of Interpersonal Problems: Psychometric properties and clinical applications. *Journal of Consulting and Clinical Psychology*, *56*, 885–892.

Horvath, A. O., & Bedi, R. P. (2002). The alliance. In J. C. Norcross (Ed.), *Psychotherapy relationships that work* (pp. 37–69). New York: Oxford University Press.

Howard, K. I., Kopta, S. M., Krause, M. S., & Orlinsky, D. E. (1986). The dose-effect relationship in psychotherapy. *American Psychologist*, *41*, 159–164.

Howard, K. I., Lueger, R. J., Maling, M. S., & Martinovich, Z. (1993). A phase model of psychotherapy outcome: Causal mediation of change. *Journal of Consulting and Clinical Psychology*, *61*, 678–685.

Howard, K. I., Orlinsky, D. E., & Lueger, R. J. (1995). The design of clinically relevant outcome research: Some considerations and an example. In M. Aveline & D. Sharpiro (Eds), *Research foundations for psychotherapy services* (pp. 3–47). Sussex, UK: Wiley.

Hyler, S., & Lyons, M. (1988). Factor analysis of the *DSM-III* personality disorder clusters: A replication. *Comprehensive Psychiatry*, *29*, 304–308.

Hyler, S., Lyons, M., Rieder, R., Young, L., Williams, J., & Spitzer, R. (1990). The factor structure of self-report *DSM-III* Axis II symptoms and their relationships to clinicians' ratings. *American Journal of Psychiatry*, *147*, 751–757.

Kass, F., Skodol, A., Charles, F., Spitzer, R., & Williams, J. B. W. (1985). Scaled ratings of the *DSM-III* personality disorders. *American Journal of Psychiatry*, *147*, 627–630.

Kolden, G. G. (1991). The generic model of psychotherapy: An empirical investigation of patterns of process and outcome relationships. *Psychotherapy Research*, *1*, 62–73.

Kolden, G. G. (1996). Change in early sessions of dynamic therapy: Universal processes and the generic model of psychotherapy. *Journal of Consulting and Clinical Psychology*, *64*, 489–496.

Kolden, G. G., & Howard, K. I. (1992). An empirical test of the generic model of psychotherapy. *The Journal of Psychotherapy Practice and Research*, *1*, 225–236.

Kolden, G. G., & Klein, M. H. (1996). Therapeutic process in dynamic therapy for personality disorders: The joint influence of acute distress and dysfunction and severity of personality pathology. *Journal of Personality Disorders*, *10*, 107–121.

Kolden, G. G., Strauman, T. J., Halverson, J., & Heerey, E. (1997, June). *Universal session-level change processes in early sessions of psychotherapy: A path model*. Presented at the international meeting of the Society for Psychotherapy Research, Geilo, Norway.

Kraus, G., & Reynolds, D. J. (2001). The "A-B-C's" of the Cluster B's: Identifying, understanding, and treating Cluster B personality disorders. *Clinical Psychology Review*, *21*, 345–400.

Lambert, M. J., Hansen, N. B., & Finch, A. E. (2001). Patient-focused research: Using patient outcome data to enhance treatment effects. *Journal of Consulting and Clinical Psychology*, *69*, 159–172.

Livesley, W. J. (Ed.). (1995). *The DSM-IV personality disorders*. New York: Guilford Press.

McNeilly, C. L., & Howard, K. I. (1991). The Therapeutic Procedures Inventory: Psychometric properties and relationship to phase of treatment. *Journal of Psychotherapy Integration*, *1*, 223–234.

Merrill, K. A., Tolbert, V. E., & Wade, W. A. (2003). Effectiveness of cognitive therapy for depression in a community mental health center: A benchmarking study. *Journal of Consulting and Clinical Psychology*, *71*, 404–409.

Millon, T. (1986). On the past and future of the *DSM-III*: Personal recollections and projections. In T. Millon & G. L. Klerman (Eds), *Contemporary directions in psychopathology: Toward the DSM-IV* (pp. 29–72). New York: Guilford Press.

Millon, T., & Klerman, G. (Eds.). (1986). *Contemporary directions in psychopathology: Toward the DSM-IV.* New York: Guilford Press.

Monsen, J. T., Odland, T., Faugli, A., Daae, E., & Eilertsen, D. E. (1995). Personality disorders: Changes and stability after intensive psychotherapy focusing on affect consciousness. *Psychotherapy Research*, *5*, 33–48.

Mulder, R. T. (2002). Personality pathology and treatment outcome in major depression: A review. *American Journal of Psychiatry*, *159*, 359–371.

Orlinsky, D. E., Grawe, K., & Parks, B. K. (1994). Process and outcome in psychotherapy—Noch einmal. In A. E. Bergin & S. L. Garfield (Eds), *Handbook of psychotherapy and behavior change* (4th ed., pp. 270–376). New York: Wiley.

Orlinsky, D. E., & Howard, K. I. (1986a). Process and outcome in psychotherapy. In S. L. Garfield & A. E. Bergin (Eds), *Handbook of psychotherapy and behavior change* (3rd ed., pp. 233–282). New York: Wiley.

Orlinsky, D. E., & Howard, K. I. (1986b). The psychological interior of psychotherapy: Explorations with the therapy session reports. In L. S. Greenberg & W. M. Pinsof (Eds), *The psychotherapeutic process: A research handbook* (pp. 477–502). New York: Guilford Press.

Orlinsky, D. E., & Howard, K. I. (1987). A generic model of psychotherapy. *Journal of Integrative and Eclectic Psychotherapy*, *6*, 6–27.

Orlinsky, D. E., Lundy, M., Howard, K. I., Davidson, C. V., & O'Mahoney, M. T. (1987). *Therapeutic Procedures Inventory–Revised*. Chicago: Northwestern University.

Orlinsky, D. E., Ronnestad, M. H., & Willutzki, U. (2004). Fifty years of psychotherapy process-outcome research: Continuity and change. In M. J. Lambert (Ed.), *Bergin and Garfield's handbook of psychotherapy and behavior change* (5th ed., pp. 307–389). New York: Wiley.

Persons, J. B., Burns, D. D., & Perloff, J. M. (1988). Predictors of dropout and outcome in cognitive therapy for depression in a private practice setting. *Cognitive Therapy & Research*, *12*, 557–575.

Pfohl, B., Black, D., Noyes, R., Coryell, W. H., & Barrash, J. (1990). Axis I/Axis II comorbidity findings. In J. M. Oldham (Ed.), *Personality disorders: New perspectives on validity* (pp. 147–161). Washington, DC: American Psychiatric Press.

Pilkonis, P. A., Heape, C. L., Ruddy, J., & Serrao, P. (1991). Validity in the diagnosis of personality disorders: The use of the LEAD standard. *Psychological Assessment*, *3*, 46–54.

Rogers, C. R. (1957). The necessary and sufficient conditions of therapeutic personality change. *Journal of Consulting Psychology*, *21*, 95–103.

Saunders, S. M., Howard, K. I., & Orlinsky, D. E. (1989). The Therapeutic Bond scales: Psychometric characteristics and relationship to treatment effectiveness. *Psychological Assessment*, *1*, 323–330.

Shea, M. T., Glass, D. R., Pilkonis, P. A., Watkins, J., & Docherty, J. P. (1987). Frequency and implications of personality disorders in a sample of depressed outpatients. *Journal of Personality Disorders*, *1*, 27–42.

Shea, M. T., Widiger, T. A., & Klein, M. H. (1992). Comorbidity of personality disorders and depression: Implications for treatment. *Journal of Consulting and Clinical Psychology*, *60*, 857–868.

Taube, C. A., Kessler, L., & Feuerberg, M. (1984). Utilization and expenditures for ambulatory mental health care during 1980. In *National medical care utilization and expenditure survey (Data Report 5)*. Washington, DC: U.S. Department of Health and Human Services.

Tyrer, P., Gunderson, J., Lyons, M., & Tohen, M. (1997). Extent of comorbidity between mental state and personality disorders. *Journal of Personality Disorders*, *11*, 242–259.

Vessey, J. T., & Howard, K. I. (1993). Who seeks psychotherapy? *Psychotherapy*, *30*, 546–553.

Wagner, C. C., Riley, W. T., Schmidt, J. A., McCormick, M. G. F., & Butler, S. F. (1999). Personality disorder styles and reciprocal interpersonal impacts during outpatient intake interviews. *Psychotherapy Research*, *9*, 216–231.

Winston, B., Winston, A., Samstag, L.W., & Muran, J. C. (1994). Patient defense/therapist interventions. *Psychotherapy*, *31*, 478–491.

Zusammenfassung

Früher Psychotherapieprozess und Cluster B und C Persönlichkeitspathologie: Ähnlichkeiten und Unterschiede in der Interaktion mit symptomatischem rund interpersonellem Stress

In einer früheren Untersuchung (Kolden & Klein, 1996) haben die Autoren gefunden, dass die Beziehungen zwischen allgemeiner Persönlichkeitspathologie und frühen Veränderungsprozessen in der Psychotherapie (wie sie im Modell der generischen Psychotherapie definiert sind) durch das Ausmaß an akutem symptomatischem und interpersonellen Stress mit beeinflusst werden. In der vorliegenden Untersuchung haben die Autoren dieselben Daten noch einmal analysiert, um Ähnlichkeiten und Unterschiede zwischen den Persönlichkeitsclustern B (dramatisierend, emotional oder unberechenbar), und C (ängstlich) im Therapieprozess aufzuzeigen. Wir fanden, dass stärker gestresste Patienten mehr Abwehr zeigten. Es gab keine signifikanten Interaktionen zwischen symptomatischem Stress und Persönlichkeitspathologie in der Vorhersage von Prozessvariablen. Patienten mit höheren Werten für Cluster B fühlten sich offener und engagierter in den Sitzungen, wenn sie zum Zeitpunkt des Beginns der Therapie weniger durch interpersonelle Probleme gestresst waren. Dagegen behinderte eine geringere interpersonelle Gestresstheit Offenheit und Einsicht bei Patienten mit hohen Werten für Cluster C. Die Therapeuten verwendeten im Allgemeinen mehr direkte Interventionen und die Exploration früherer Erfahrungen bei der Arbeit mit Patienten mit hohen Werten für Cluster C. Ganz spezifisch jedoch gebrauchten die Therapeuten direkte Interventionen häufiger im Falle schwerer gestörter Cluster B Patienten mit hohem interpersonellem Stress. In der Diskussion wird auf die Implikationen der Erleichterung eines produktiven frühen Therapieprozesses bei Patienten mit Persönlichkeitsstörungen vom Typ der Cluster B und C eingegangen.

Résumé

Processus psychothérapeutique précoce et pathologies de personnalité Cluster B et C : Ressemblances et différences dans les interactions avec la détresse symptomatique et interpersonnelle

Dans une étude précédente (Kolden & Klein, 1996), les auteurs ont trouvé que les relations entre la pathologie de personnalité globale et les processus de changement précoces en psychothérapie (comme définis par le modèle générique de la psychothérapie) étaient modérées par le degré de la détresse symptomatique et interpersonnelle aigue des patients. Dans l'étude présente, les auteurs ont repris les mêmes données pour examiner des ressemblances et des différences du processus thérapeutique entre les troubles de personnalité des clusters B (dramatiques, émotionnels ou erratiques) et C (anxieux ou craintifs). En général, nous avons trouvé que les patients plus perturbés rapportaient une attitude plus défensive. Il n'y avait d'interaction significative entre la détresse symptomatique et la pathologie de personnalité pour aucune des variables du processus. Cependant, la détresse interpersonnelle modérait les relations entre les clusters B et C et certains processus thérapeutiques. Les patients à haute présence de cluster C se sentaient plus ouverts et impliqués dans la séance lorsqu'ils étaient moins perturbés par leurs problèmes interpersonnels au départ de la thérapie. Par contre, l'ouverture et l'insight étaient entravés parmi les patients à haute présence de cluster B lorsqu'ils étaient moins perturbés sur le plan interpersonnel. Quant aux thérapeutes, ils ont, en règle générale utilisé plus d'interventions directes et d'exploration d'expériences passées en travaillant avec des patients à plus haute pathologie de cluster C. Toutefois, les thérapeutes ont utilisé de façon plus spécifique les interventions directes lorsque des patients avec une pathologie de cluster C plus sévère accusaient aussi une plus grande détresse interpersonnelle. La discussion considère des implications en vue d'une facilitation d'un processus thérapeutique précoce productif chez des patients souffrant d'une pathologie des clusters B ou C.

Resumen

Proceso psicoterapéutico temprano y patología de personalidades de los clusters B y C. semejanzas y diferencias en las interacciones con distrés sintomático e interpersonal

En un estudio previo (Kolden & Klein, 1996), los autores encontraron que las relaciones entre la patología de la personalidad global y los procesos de cambio temprano en la psicoterapia (según definición del modelo genérico de psicoterapia) fueron atemperados por la magnitud del distrés sintomático e interpersonal agudo del paciente. En el estudio actual, los autores reanalizan los mismos datos para examinar las semejanzas y diferencias entre los

desórdenes de personalidad de los Clusters B (dramático, emocional o errático) y C (ansioso o temeroso) en el proceso terapéutico. En general, encontraron que los pacientes más distresados registraron una mayor defensividad. No hubo interacciones significativas entre el distrés sintomático y la patología de la personalidad en la predicción de ninguna variable del proceso. Sin embargo, el distrés interpersonal atemperó las relaciones entre los Clusters B y C y algunos procesos terapéuticos. Los pacientes con valores altos en Cluster B se sintieron más abiertos e involucrados en la sesión cuando al comienzo de la terapia estaban menos distresados por sus problemas interpersonales. En contraste, la apertura y el insight estuvieron inhibidos entre los pacientes con valores altos en el Cluster C cuando estuvieron menos distresados interpersonalmente. Los terapeutas emplearon, generalmente, intervenciones y exploraciones más directas de experiencias pasadas cuando trabajaron con pacientes de mayor patología en cluster C. Sin embargo, los terapeutas usaron más específicamente intervenciones directas cuando los pacientes con patología más severa en Cluster B también puntuaron alto en distrés interpersonal. La discusión considera implicaciones para la facilitación de procesos productivos tempranos en terapia de pacientes que sufren de patología en los Clusters B o C de personalidad.

Resumo

Processos Psicoterapêuticos Precoces e Psicopatologia da Personalidade de Cluster B e C: Semelhanças e Diferenças na Interacção com Perturbação Sintomático e Distress Interpessoal

Num estudo anterior (Kolden & Klein, 1996), os autores encontraram que as relações entre patologia global da personalidade e os processos de mudança psicoterapeutica precoces (tal como definidos pelo modelo genérico de Psicoterapia) foram moderados pelo nível de perturbação sintomática e perturbação interpessoal dos pacientes. No presente estudo, os autores re-analisaram estes dados para examinar as semelhanças e diferenças entre as perturbações da personalidade do cluster B (dramático, emocional ou errático) e C (ansioso ou medroso) no processo terapêutico.

Em geral, os pacientes mais perturbados relatavam maior defensividade. Não se verificaram interacções significativas entre o nível de perturbação e a patologia da personalidade na predição de qualquer uma das variáveis processo. Porém, a perturbação interpessoal modera as relações entre o cluster B e C e alguns processos terapêuticos. Os pacientes com resultados elevados no cluster B sentiam maior abertura e envolvimento na sessão quando eles estavam menos perturbados com os seus problemas interpessoais, no início da terapia. Em contraste, abertura e insight estavam comprometidos nos pacientes com níveis elevados no cluster C quando menos perturbados em termos interpessoais. Os terapeutas em geral usaram mais intervenções directas e exploração de experiências passadas quando trabalhavam com pacientes com patologia elevada do cluster C. Porém, os terapeutas usavam intervenções directas mais especificamente quando os pacientes com maior patologia do cluster B também tinham elevado nível de perturbação interpessoal. A discussão considera implicações para a facilitação de processos iniciais produtivos na terapia precoce em pacientes que sofrem de patologia de personalidade do Cluster B ou C.

Sommario

In uno studio precedente (Kolden, Klein, 1996), gli autori hanno trovato che la relazione tra la patologia globale della personalità e I cambiamenti nelle prime fasi della terapia (così come definito dal modello generico della psicoterapia) è determinate dal livello di sintomatologia acuta e di sofferenza interpersonale.. Nel presente studio, gli autori hanno riesaminato i medesimi dati per valutare le differenze e le similitudini nel processo terapeutico In generale si è potuto notare come i pazienti più sofferenti fossero caratterizzati da un maggior uso di difese. Non ci sono state interazioni significative tra la sofferenza sintomatologia e la patologia di personalità rispetto alle variabili di processo. Inoltre le problematiche interpersonali hanno mostrato relazioni moderate tra i cluster B e C e alcune variabili processuali. I pazienti del cluster B si sono sentiti più coinvolti nelle loro sedute quando erano meno stressati dai loro problemi interpersonali al principo della terapia. In contrasto, l'apertura e l'insight erano impediti tra i pazienti del cluster C quando erno meno stressati dai problemi interpersonali. I terapeuti in generale hanno utilizzato maggiormente interventi più diretti ed esplorato maggiormente l'esperieze passate con I pazienti del cluster C. Comunque i terapeuti hanno usato interventi diretti anche con i pazienti più gravi del Cluster B quando erano stressati da problematiche interpersonali. La discussione considera implicazioni per facilitare il processo terapeutico nelle prime fasi della terapia.

摘要

在先前的一篇研究(Kolden & Klein, 1996)中，研究者發現整體的人格病狀和初期的心理治療改變歷程（這是指一般的心理治療模式而言），二者的關係是受到個案症狀的急性程度以及人際關系的困擾程度而定。本研究係採用相同的資料，檢視人格違常的B群（戲劇性、情緒性和乖僻性）和C群（焦慮與恐懼）的相似或相異性。整體而言，研究者發現困擾程度越高的病患，有越多的防衛。至於用症狀的困擾程度以及人格的病狀二方面的交互作用來預測任何歷程變項則均無顯著差異。不過人際關係的困擾程度可調節B群和C群及一些治療歷程。在B群得分高的病患，若是治療初期較少人際困擾，則在治療期間會表現較為開放且較為投入。反之，C群得分高的病患，若是他們較少人際困擾，反而會阻礙其在治療期間的開放度與頓悟程度。治療師對於C群病症程度高的病患，通常採用較直接的處遇策略，並且會探索過去的經歷。不過治療師對於較嚴重的B群病症且高度人際困擾的病患，亦會運用直接介入的策略。研究討論主要係針對B群與C群病患，應該如何催化，才能有助於治療初期階段的進展。